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Subnational implementation review workshop of the Big Catch-Up (BCU) immunization initiative in priority countries of the WHO South-East Asia Region

10–12 December 2025, New Delhi, India

Report of a workshop

World Health Organization
South-East Asia Region

Subnational implementation review workshop of the Big Catch-Up (BCU) immunization initiative in priority countries of the WHO South-East Asia Region

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Abbreviations and acronyms

AEFI	adverse event following immunization
AFP	acute flaccid paralysis
ANM	auxiliary nurse midwife
ASHA	Accredited Social Health Activist
AWPB	Annual Workplan and Budget
BCG	Bacille Calmette-Guérin
BCU	Big Catch-Up
BeSD	behavioural and social drivers
CCEOP	Cold Chain Equipment Optimization Platform
CHV	community health volunteer
CSO	civil society organization
DHIS2	District Health Information System 2
DPT	diphtheria, pertussis, tetanus
DTP3	third dose of diphtheria–tetanus–pertussis vaccine
DTPCV	diphtheria–tetanus–pertussis-containing vaccine
DQA	data quality assessment
DQS	data quality self-assessment
EAF	Equity Accelerator Fund (Gavi, the Vaccine Alliance)
eLMIS	electronic logistics management information system
eJRF	electronic Joint Reporting Form
EPI	Expanded Programme on Immunization
eVIN	electronic Vaccine Intelligence Network
EVM	effective vaccine management
FCHV	female community health volunteer
Gavi	Gavi, the Vaccine Alliance
GIS	geographic information system
HepB	hepatitis B
HepB-BD	hepatitis B birth dose
HIS	health information system
HMIS	health management information system
HPV	human papillomavirus
HPVc	final dose of HPV vaccine
HR	human resources
HSS	health system strengthening
IA2030	Immunization Agenda 2030
IDP	internally displaced person
IDSP	Integrated Disease Surveillance Programme
IEC	information, education and communication
IMI	Intensified Mission Indradhanush
IPC	infection prevention and control
IPV	inactivated poliovirus vaccine
IPV2	second dose of inactivated poliovirus vaccine
ITSU	Immunization Technical Support Unit
IVD	Immunization and Vaccine Development Unit
JE	Japanese encephalitis

JSI	John Snow India
KAP	knowledge, attitudes and practices
LMICs	low- and middle-income countries
M&E	monitoring and evaluation
MCV	measles-containing vaccine
MCV1	first dose of measles-containing vaccine
MCV2	second dose of measles-containing vaccine
MCP Card	Mother and Child Protection Card
MEL	monitoring, evaluation and learning
MI	Mission Indradhanush
MNTE	maternal and neonatal tetanus elimination
MOV	missed opportunities for vaccination
MR	measles–rubella vaccine
MR1	first dose of measles–rubella vaccine
MR2	second dose of measles–rubella vaccine
NIP	National Immunization Programme
NIS	National Immunization Strategy
NITAG	National Immunization Technical Advisory Group
NMNR	non-measles, non-rubella
NPSN	National Polio Surveillance Network
PATH	Program for Appropriate Technology in Health
PCV	pneumococcal conjugate vaccine
PCV3	third dose of pneumococcal conjugate vaccine
Penta	pentavalent vaccine
Penta3	third dose of pentavalent vaccine
PHC	primary health care
PHEIC	public health emergency of international concern
PIRI	periodic intensification of routine immunization
PIS	Programa Integradu Saúde (Timor-Leste)
RI	routine immunization
SBC	social and behaviour change
SIA	supplementary immunization activity
SOP	standard operating procedure
tOPV / bOPV	trivalent oral polio vaccine/bivalent oral polio vaccine
TCA	Targeted Country Assistance (Gavi)
UIP	Universal Immunization Programme
U-WIN	Universal Immunization Web-enabled Network
UNICEF	United Nations Children's Fund
VPOP	Vaccine Portfolio Optimization and Prioritization
WHO	World Health Organization
WHO-SEARO	WHO Regional Office for South-East Asia
WHO HQ	WHO headquarters
WUENIC	WHO-UNICEF estimates of national immunization coverage

1. Executive summary

The WHO Regional Office for South-East Asia (WHO-SEARO) convened the subnational implementation review workshop of the Big Catch-Up (BCU) immunization initiative in New Delhi on 10–12 December 2025, bringing together national and subnational immunization managers, technical experts and WHO staff members from Nepal, Myanmar, India and Timor-Leste, alongside WHO personnel from the Regional Office and headquarters. Key partners – including the United Nations Children's Fund (UNICEF), Gavi, the Vaccine Alliance (Gavi), Program for Appropriate Technology in Health (PATH), John Snow India (JSI) and Immunization Technical Support Unit (ITSU) – joined the workshop to jointly review BCU progress and shape strategies for institutionalizing catch-up vaccination.

The workshop highlighted significant recovery since the launch of the BCU initiative in 2023 to address immunity gaps widened by COVID-19. Countries reported meaningful reductions in zero-dose children, improved antigen coverage and increasing integration of catch-up into routine systems. India's progress stood out, with a 43% reduction in zero-dose children and strong antigen-specific gains supported by digital platforms such as the Universal Immunization Web-enabled Network (U-WIN).

Throughout the workshop, country teams exchanged lessons on addressing persistent routine immunization gaps, leveraging digital tools for tracking and monitoring, and tailoring strategies for urban, mobile and hard-to-reach populations. Field visits in Delhi provided participants with practical exposure to service delivery models, community engagement approaches and local innovations that are helping to strengthen coverage.

Technical sessions from WHO and partners reinforced these discussions. WHO emphasized critical data gaps and the need for stronger digital systems to sustain routine catch-up. The WHO headquarters (WHO HQ) provided global insights, noting that while the BCU initiative improved planning, monitoring and stakeholder engagement, countries must now transition from campaign-style activities to fully institutionalized routine systems.

Partners presented complementary perspectives. Gavi outlined its 6.0 strategy to streamline funding, improve data systems and scale innovations. UNICEF presented regional insights and case studies from Nepal and Myanmar, underscoring the role of community engagement, behavioural insights, and strengthened cold-chain and delivery systems in improving equity and sustaining routine immunization.

India's Delhi team demonstrated how targeted microplanning, U-WIN-enabled tracking and focused outreach helped reduce zero-dose children in complex urban settings. PATH showcased a range of innovations – from cold-chain technologies to

human-centered design and digital tools – that can improve access for underserved populations. ITSU highlighted the need for stronger urban-focused planning, enhanced microplanning and data-driven, community-centered delivery approaches. The JSI emphasized the importance of localization through community-based organizations and hyper-local communication to improve vaccination uptake and equity.

Despite progress, participants acknowledged ongoing challenges, including coverage inequities, data quality issues, resource constraints and workforce limitations. Countries identified key priorities: developing country-owned workplans, strengthening real-time data systems, sustaining partner engagement, and prioritizing approaches for vulnerable and underserved populations.

Closing the workshop, Dr Suman Rijal, Director for Department of Communicable Diseases, WHO-SEARO, underscored the need to embed catch-up vaccination as a routine, continuous component of immunization programmes. He emphasized prioritizing vulnerable groups, smarter resource allocation, stronger community engagement and accelerated digitalization to sustain gains. He reaffirmed WHO's commitment to continued collaboration and cross-country learning to ensure no child is left behind.

The BCU workshop served as a critical regional platform to consolidate evidence, share practical solutions and reinforce collective commitment to institutionalizing catch-up vaccination across the Region.

2. Background

The Big Catch-Up (BCU) initiative was launched globally in April 2023 as an urgent response to the unprecedented immunization setbacks caused by the COVID-19 pandemic. During the pandemic, routine immunization services were severely disrupted due to lockdowns, supply chain interruptions and diversion of health resources to COVID-19 response efforts. As a result, millions of children worldwide missed essential vaccines, creating significant immunity gaps and increasing the risk of outbreaks of vaccine-preventable diseases (VPDs), such as measles, polio and diphtheria.

Recognizing this critical threat to global health security, WHO, UNICEF, Gavi and other partners came together to introduce the Big Catch-Up as a coordinated, high-impact initiative. The global call to action emphasized the need to restore immunization coverage to pre-pandemic levels, accelerate catch-up vaccination for zero-dose and under-immunized children, and strengthen routine immunization systems to prevent future disruptions. The initiative was aligned with the Immunization Agenda 2030 (IA2030), reinforcing its strategic vision of "leaving no one behind".

The BCU aimed to restore immunization coverage to pre-pandemic levels, accelerate catch-up vaccination for zero-dose and under-immunized children, and strengthen routine immunization systems for long-term resilience. Its strategic focus included:

- closing immunity gaps among children aged 12–59 months;
- embedding catch-up activities into routine programmes;
- mobilizing global and regional partnerships for technical and financial support; and
- driving advocacy and communication to raise awareness.

WHO provided technical guidance, monitoring frameworks, and tools to help countries plan and track progress effectively.

In the WHO South-East (SE) Asia Region, Myanmar and Nepal received Gavi support and launched national BCU campaigns in 2024. These campaigns involved multiple rounds of outreach, integration with primary health care and enhanced monitoring systems. The last rounds were completed in May 2025, but unused vaccine doses remain, requiring further planning for effective utilization in future catch-up and routine immunization activities. Both countries have been submitting quarterly progress reports to WHO-SEARO, WHO HQ and Gavi.

The BCU initiative was scheduled to conclude in December 2025, making this workshop timely and essential. It provided an opportunity for countries to:

- review full-year implementation progress;
- consolidate lessons learnt; and
- develop strategies to institutionalize catch-up vaccination as a routine component of immunization programmes.

In addition, Timor-Leste and India were included in the BCU review workshop to enrich regional learning and foster practical exchange of experiences. Timor-Leste, while not a Gavi-supported BCU country, has implemented targeted catch-up activities and demonstrated innovative approaches to reach remote populations, offering valuable lessons for other small and resource-constrained settings.

India, on the other hand, has successfully integrated large-scale government-funded catch-up campaigns into its routine immunization system and leveraged digital platforms such as U-WIN for real-time monitoring. Including these two countries provided an opportunity for participants to learn from diverse contexts – ranging from small island nations to large federal systems – ensuring that strategies discussed during the workshop are adaptable and scalable across different country settings.

3. Objectives

The overall objective of the BCU review workshop was to assess the implementation of the BCU initiative in priority countries of the WHO SE Asia Region by conducting a comprehensive subnational review, sharing lessons learnt, and developing tailored strategies to institutionalize catch-up vaccination and optimize resource allocation in the context of reduced partner support.

Specific objectives were to:

- conduct a detailed subnational analysis of BCU implementation in Nepal and Myanmar as well as local resource-funded catch-up activities in India, focusing on:
    - identification of un- and under-immunized children at subnational levels;
    - conduct of catch-up immunization activities, including outreach and service delivery strategies;
    - reporting and validation of coverage outcomes, with attention to data quality and completeness;
    - assessment of remaining immunization gaps, particularly among high-risk or hard-to-reach populations;
    - documentation of successful strategies that contributed to improved coverage and equity; and
    - analysis of challenges encountered during implementation and approaches used to address them.
- share lessons learnt and effective practices from BCU counties and India to sustain the recovery momentum;
- strengthen evidence-based planning and decision-making for institutionalizing catch-up immunization through improved use of subnational data; and
- prioritize corrective actions and resource allocation in response to reduced partner and donor support.

4. Proceedings of the workshop

Opening session

The BCU review workshop opened with a warm welcome extended to all participants, setting a collaborative and forward-looking tone for the three-day event. In his opening remarks, Dr Vinod Bura, Coordinator, Immunization and Vaccine Development (IVD) Unit, WHO-SEARO, emphasized the significance of the Big Catch-Up initiative as a collective effort to close immunity gaps widened during recent years and to reinforce routine immunization (RI) systems. Participants were reminded that the workshop served not only as a platform to reflect on progress, but also as an opportunity to shape a sustainable path towards institutionalizing catch-up vaccination within national programmes.

The session highlighted the strong commitment and partnership underpinning the initiative, acknowledging the essential contributions of technical teams, partner organizations and stakeholders working across different levels of the health system. Key messages underscored both the notable gains achieved through catch-up efforts and the persistent challenges – such as remaining zero-dose pockets, resource constraints, data quality issues and workforce pressures – that continue to require coordinated action.

Following the remarks, the workshop objectives were outlined, orienting participants to the core purpose of the meeting: conducting an in-depth subnational review of Big Catch-Up implementation; sharing lessons learnt and successful strategies; strengthening evidence-based planning through improved use of subnational data; and prioritizing corrective actions and resource allocation in the context of reduced partner support. This framing provided a clear roadmap for the workshop ahead and reinforced the collective responsibility to accelerate recovery and ensure no child is left behind.

Session 1: Country achievements on BCU at the national and subnational levels (2024–2025)

India

The India presentation provided a comprehensive overview of the country's immunization landscape and highlighted the critical role of the catch-up activities in accelerating recovery of routine immunization following recent disruptions. It began with a geographical and demographic profile underscoring India's vast scale, complex terrain and population diversity – factors that shape service delivery and contribute to persistent immunization inequities.

With an estimated population of 1.4 billion and significant urban–rural mobility, presenters noted the need for tailored immunization strategies to reach migrant, remote and high-risk communities.

A detailed review of the Universal Immunization Programme (UIP) illustrated India's longstanding commitment to vaccine-preventable disease control. As one of the world's largest immunization programmes, UIP executes nearly 14 million vaccination sessions annually and supports 25.5 million newborns and 30 million pregnant women each year. The presentation traced major immunization milestones, from polio eradication and elimination of maternal and neonatal tetanus to the introduction of multiple new vaccines and deployment of digital tools such as U-WIN, which enabled name-based tracking and strengthened logistics and data management.

The impact of the catch-up activities was a central theme. India demonstrated notable gains in reducing zero-dose and under-immunized children, with pre-pandemic coverage levels restored and, in some cases, surpassed. Data highlighted substantial improvements in DTP1, DTP3 and Measles-containing vaccine coverage, as well as a reduction of nearly one million measles zero-dose children between 2023 and 2024. Field experiences from campaign districts illustrated how targeted outreach, school-based efforts and outbreak-responsive immunization contributed to closing immunity gaps.

India also showcased strong programmatic innovations and best practices developed through intensified immunization campaigns such as Mission Indradhanush (MI) and its subsequent phases. These included microplanning for underserved pockets, community engagement initiatives, multichannel communication campaigns, and capacity-building at state, district and block levels. The presentation emphasized the importance of task force mechanisms, digitalization and intersectoral coordination in improving service quality and accountability.

Despite progress, India acknowledged persisting challenges, particularly pockets of zero-dose children, subnational inequities, high dropout rates in some settings and variations in coverage across socioeconomic groups. Rapid urbanization, population movement and the risk of accumulating susceptible cohorts were highlighted as ongoing concerns requiring sustained vigilance and adaptive strategies.

The presentation concluded by outlining how India plans to leverage the catch-up experience beyond 2026. This includes periodic risk categorization, targeted response campaigns following outbreaks, antigen-specific catch-up drives and continued integration of catch-up strategies into routine immunization systems. The overarching message emphasized the importance of sustained data-driven action, community engagement and institutionalization of catch-up mechanisms to safeguard hard-earned gains and ensure equitable vaccination coverage across the country.

Myanmar

The Myanmar presentation offered a detailed and candid overview of the country's progress, challenges and forward-looking strategies in implementing the BCU initiative amid a uniquely complex operational environment. Beginning with demographic context, the presenters highlighted Myanmar's sizeable population of children under one and under five years of age, its predominantly rural settlement patterns, and its highly diverse administrative landscape of 17 states/regions and more than 13 000 village tracts. This backdrop framed the operational realities of delivering immunization services across geographically difficult and security-compromised areas.

The review of Myanmar's current immunization strategies emphasized the integration of routine, catch-up and outbreak-response activities. Special immunization sessions targeted children in monasteries, nunneries, orphanages, internally displaced people (IDP) camps and hard-to-reach communities, while outbreak response immunization remained essential for control of acute flaccid paralysis (AFP), measles and diphtheria. Myanmar's area-based planning approach – classifying townships into A, B and C categories based on resource availability, security and accessibility – emerged as a key enabling factor, allowing phased implementation tailored to contextual constraints. However, it also underscored persistent barriers in "C" areas where security issues, limited vaccinators and non-functional health facilities have delayed BCU progress.

The review detailed BCU planning and operational guidelines, including the phased rollout of vaccine schedules, the addition of Japanese encephalitis (JE) vaccine in later phases and the progressive strengthening of national standard operating procedures (SOPs). Despite supply chain achievements – such as microplanning, vaccine demand forecasting and operational support for transport – vaccine stockouts, high transport costs and global shortages of certain antigens were noted as challenges. The presenters also emphasized the importance of integrating catch-up vaccine forecasting into broader annual planning to improve future preparedness.

Capacity-building efforts formed a major component of Myanmar's BCU implementation. More than 880 health workers received training through cascade models, supported by standardized SOPs, flowcharts and job aids, designed to ensure accurate eligibility assessments and proper vaccine administration practices. While general competency among health workers has improved, refresher trainings and continued supportive supervision remain essential, particularly for newly trained vaccinators and staff in remote locations.

Myanmar's investments in recording and reporting systems were also highlighted. The adaptation of District Health Information System 2 (DHIS2) for online catch-up

reporting, alongside revised vaccination cards and daily reporting forms, has enabled better data separation between routine and catch-up doses. Yet, real-time data gaps and uneven adoption in inaccessible areas indicate the need for further digital strengthening and investment in user-friendly tools.

Community engagement and demand generation strategies were central to Myanmar's BCU experience. The development of information, education and communication (IEC) materials, household mobilization, and engagement with community leaders helped increase caregiver awareness and build trust. These approaches improved uptake, but misinformation, inconsistent connectivity and limited time for outreach hindered efforts in some regions. The way forward emphasized deepening local media use, reinforcing infection prevention and control (IPC) skills among health workers, and embedding demand-generation activities into microplanning cycles.

The review also demonstrated measurable BCU impact. Between 2021 and 2024, Myanmar reached over 1.2 million zero-dose children, an impressive achievement given operational constraint. Township-wise mapping displayed growing geographical coverage over successive years, while national coverage indicators for oral polio vaccine (OPV), pentavalent vaccine (Penta), measles–rubella vaccine (MR) and inactivated poliovirus vaccine (IPV) showed steady improvements. Nonetheless, persistent zero-dose pockets remain, particularly in mobile and conflict-affected populations where missed opportunities continue to widen inequities.

Key lessons learnt included the effectiveness of phased planning, the essential role of community volunteers and NGOs, the advantages of integrating immunization with nutrition and other health services, and the value of strengthened data systems. These insights informed the country's roadmap for 2026 and beyond, which focuses on continuing BCU in pending townships, revising strategies for gap areas, institutionalizing delayed-dose SOPs, expanding partnerships with local actors, securing sustainable financing and enhancing multisectoral collaboration.

Overall, the Myanmar review reflected a strong commitment to recovering and sustaining immunization gains despite profound structural and contextual challenges. Through adaptive planning, community-centred approaches and continuous systems strengthening, Myanmar aims to ensure equitable access to life-saving vaccines and protect vulnerable children across all regions.

Nepal

The Nepal presentation provided a comprehensive and data-rich account of the country's progress, lessons and future directions in implementing the BCU initiative within the broader National Immunization Programme (NIP). The review opened with an overview

of Nepal's demographic landscape, highlighting its population of nearly 30 million, diverse geography spanning mountains to plains regions and an under-five population exceeding 2.3 million. These demographic and geographical realities frame both the opportunities and operational challenges for immunization delivery across seven provinces, 77 districts and 753 municipalities.

A central focus of the review was Nepal's strong recovery in routine immunization coverage following pandemic-related disruptions. National data showed consistent improvement across key antigens from 2019 to 2024, culminating in the highest-ever coverage in 2024, according to WHO-UNICEF estimates of national immunization coverage (WUENIC). For example, Penta1, Penta3, MR1, MR2, IPV and third dose of pneumococcal conjugate vaccine (PCV3) all reached coverage levels ranging from 93% to 99%, reflecting a significant rebound and a narrowing of historical immunity gaps. Penta3 coverage maps illustrated year-on-year gains in subnational performance, with many districts crossing the 90% threshold by 2024. The number of zero-dose and missed-dose children for Penta and MR vaccines also showed marked declines, compared with pre-pandemic years.

Nepal attributed this progress to a combination of strategic and programmatic interventions. Routine immunization strengthening included expanding eligibility for delayed vaccination up to five years of age, revising immunization schedules, dedicating months to intensive vaccination and advocacy (such as "Immunization Month"), and institutionalizing the Full Immunization Declaration and Sustainability initiative. Microplanning improvements, targeted outreach in hard-to-reach communities, and strong engagement of female community health volunteers (FCHVs) – a trusted workforce embedded in the community – were highlighted as critical enablers of success.

The review of BCU-specific components outlined Nepal's policy environment, including the Immunization Act (2016), regulatory updates and detailed SOPs for catch-up vaccination. The country developed a clear schedule for missed or delayed doses, ensuring that children up to five years old could be fully vaccinated regardless of previous contact with immunization services. Vaccinees in all age-eligible groups benefited from a strengthened supply chain, supported through Gavi for key antigens (Penta and IPV) and complemented by Government of Nepal procurement and multiyear vaccine forecasting mechanisms.

Health worker capacity was another key pillar. Nepal shared data from field monitoring, showing high levels of competency among health workers in assessing vaccination status, determining eligibility for catch-up, administering multiple injections safely, and using new tools and guidelines. Monitoring findings from 2024–2025

demonstrated that 95%–98% of facilities and session sites adhered to recommended practices. However, the review acknowledged the need for refresher training, continued supportive supervision and simplified guidelines for new staff.

On recording and reporting, Nepal acknowledged both strengths and system gaps. While DHIS2 and immunization registers include provisions for children starting vaccinations after age two, antigen-wise reporting of delayed doses for those aged 24–59 months is currently maintained through standalone Excel systems not yet integrated into routine health management information system (HMIS) and DHIS-2. Addressing this gap was underscored as a priority to institutionalize catch-up reporting within national systems.

Demand generation and community engagement formed a major theme throughout the reviewal presentation. Nepal showcased a broad communication effort, including the development of IEC materials, social media campaigns, interpersonal communication strengthening for FCHVs, and multistakeholder advocacy involving pediatricians, professional associations, religious networks, journalists and local elected representatives. These activities aimed to build trust, counter misinformation and ensure caregivers were aware of opportunities for missed-dose vaccination. Catch-up messaging was integrated into the well-established biannual vitamin A campaigns, further expanding reach.

The review also outlined strategies for 2025/2026 that operationalize Nepal's commitment to "leave no one behind", These include household surveys in high-risk districts, strengthened microplans targeting migrant, border and underserved populations, additional fixed vaccination days in metropolitan areas, mobile clinics in remote Himalayan districts and increased advocacy across subnational administrative levels. These strategies align with Nepal's extensive decentralization, ensuring local governments co-own immunization responsibilities.

In terms of financing, the review noted a significant increase in the Government of Nepal funding for NIP – from 36% to 55% between FY 2024/2025 and FY 2025/2026 – as external partner contributions decline. While funding is secured for BCU-supportive activities until December 2025, shortages are anticipated from January 2026 onwards, requiring increased domestic resource mobilization and strategic prioritization.

Key lessons emphasized that the Big Catch-Up initiative has been instrumental in closing immunity gaps and accelerating Nepal's post-pandemic recovery. However, sustaining gains will require embedding catch-up activities within routine immunization, strengthening data systems for delayed vaccination reporting, continuously engaging communities and ensuring consistent funding. The review concluded with a

forward-looking action plan for 2026 and beyond, prioritizing full implementation of all activities planned in the Annual Workplan and Budget (AWPB), strengthening monitoring systems, sustaining advocacy and reinforcing the vital role of FCHVs in reaching every child.

Timor-Leste

The Timor-Leste presentation offered a structured and insightful overview of the country's progress, challenges, and future plans for strengthening routine and catch-up immunization. The review began with a contextual background outlining national priorities for immunization within the Expanded Programme on Immunization (EPI) and highlighting the role of the Programa Integradu Saúde (PIS), an integrated primary health care platform that promotes community-level service delivery, including vaccination.

The reviewal presentation introduced Timor-Leste's health system infrastructure, which includes a national hospital, five referral hospitals, more than 70 community health centres and over 350 health posts, supported by outreach and mobile clinics to reach remote *suco* (villages). With a national population of approximately 1.4 million including more than 40 000 infants and over 170 000 children under five, the review underscored the importance of equitable access to immunization services in a country characterized by challenging terrain and dispersed communities.

The evolution of Timor-Leste's immunization programme was mapped across key milestones: introduction of new vaccines, elimination of maternal and neonatal tetanus, transition from trivalent oral polio vaccine (tOPV) to bivalent oral polio vaccine (bOPV), external reviews, and certification achievements in measles and rubella control. The recent introduction of human papillomavirus (HPV) vaccine and ongoing strategic planning for 2026–2030 demonstrate the country's continued commitment to protecting all children against vaccine-preventable diseases.

Timor-Leste shared five-year coverage trends (2020–2024) based on WUENIC, showing steady but modest coverage levels for key antigens such as BCG, Penta3, OPV3 and MCV1/MCV2. Additional administrative data for under-one-year-olds illustrated year-to-year fluctuations influenced by population shifts and service delivery constraints. The review also included data on catch-up immunization among children over one year of age, showing increases in the number of doses administered for polio, pentavalent and MR vaccines, particularly in 2023 and 2024.

A national immunization schedule was presented, with a clear demarcation of recommended versus acceptable delayed vaccination opportunities across age groups. Timor-Leste emphasized its strategy to reach zero-dose and under-immunized children through a combination of fixed-site services, outreach sessions, Servisu Integradu da

Saude Communitaria (SISCa) community health days, Programa Integradu Saúde (PIS-Integrated Health Services) activities, mobile clinics and house-to-house visits. Community engagement was strengthened by involving local leaders, teachers, religious figures and community health volunteers (CHVs), reflecting a holistic, community-driven approach.

Immunization financing included a mix of state budget and Gavi health systems strengthening support. However, resource limitations remain a significant barrier. The review highlighted major challenges affecting coverage and service delivery: limited operational budgets, transport and logistics constraints, ageing cold-chain equipment, data quality issues, staff shortages and turnover, vaccine stockouts, denominator uncertainties and external shocks such as a recent rabies outbreak.

Looking ahead, Timor-Leste outlined a pragmatic way forward focused on integrated microplanning, strengthening of financial disbursement mechanisms, ongoing supportive supervision and training, improved data quality assessment and sustained partner support to fill operational gaps. Additional focus areas included human resource retention policies, training for managers, integration of immunization activities with supplemental immunization activities (SIAs), and implementation of recommendations from effective vaccine management (EVM) and EPI reviews.

Overall, the review reflected Timor-Leste's strong commitment to advancing routine and catch-up immunization, despite systemic constraints. Through community-based outreach, integrated primary health care strategies and strengthened programme management, the country aims to further improve immunization equity and ensure life-saving vaccines reach every child.

Session 2: Overview and partner perspectives

Overview of Big Catch-Up and institutionalizing catch-up immunization – WHO SEARO

WHO-SEARO presented a regional overview of the Big Catch-Up (BCU) initiative, highlighting its rationale, regional progress and the shift towards institutionalizing catch-up immunization. It underscored the sharp immunity gaps caused by COVID-19 disruptions, including the SE Asia Region's rise in zero-dose children from 2 million in 2019 to 4.5 million in 2021. BCU efforts aim not only to restore pre-pandemic coverage but also to accelerate progress towards global goals under Gavi 5.0 and IA2030.

Implementation varies across countries: Nepal and Myanmar conducted full Gavi-supported BCU campaigns; the Democratic People's Republic of Korea (DPR Korea) used routine doses under Gavi approval; India, Indonesia, Bangladesh and Timor-Leste

carried out domestically funded catch-up efforts; and Sri Lanka, Thailand, Bhutan and Maldives focused on targeted strategies. All 11 SE Asia Region countries engaged in BCU activities in some form. Notable achievements include Nepal's 740 000 catch-up doses, Myanmar's 2.4 million doses despite access constraints, DPR Korea's 3.4 million doses through multi-antigen campaigns, and India's significant reduction in zero-dose children and improved coverage for key antigens.

Regional impact indicators show improvement: the number of zero-dose population fell from 2.5 million in 2023 to 1.9 million in 2024, while several countries neared or exceeded 90% coverage for major antigens. However, Myanmar, Indonesia and Timor-Leste remain below this benchmark due to persistent service delivery and access barriers.

The regional review emphasized remaining challenges, including subnational inequities, weak data systems, socioeconomic barriers, insufficient strategies for migrant and mobile populations, and declining partner support. It highlighted the need to institutionalize catch-up immunization by embedding it into routine systems through supportive policies, flexible schedules, adapted data platforms, sufficient supply and diversified service delivery modalities.

WHO-SEARO concluded by encouraging regional learning, reflection and collective efforts to strengthen equitable and resilient immunization systems, using the momentum of BCU to advance long-term routine immunization improvements.

UNICEF insights on implementation and strengthening routine immunization

UNICEF presented the BCU as a three-pronged effort to reach missed children, restore coverage to at least 2019 levels and strengthen immunization systems within primary health care to resume the IA2030 trajectory. Support across the SE Asia Region focused on vaccine supply and dose reconciliation, social and behaviour change (SBC), community engagement, frontline worker training and system strengthening.

UNICEF reported over 220 million BCU doses shipped globally, with 4.9 million delivered to three SE Asia Region countries, supported by optimized supply planning, phased deliveries, cold chain readiness and last-mile logistics. Strong forecasting and supplier engagement helped maintain supply stability as countries entered the dose reconciliation phase.

SBC was highlighted as central to equitable coverage. Country offices conducted behavioural assessments [knowledge, attitudes and practices (KAP), behavioural and social drivers (BeSD), gender analyses, social listening] and strengthened interpersonal communication and human-centred design approaches among frontline workers.

Partnerships with community leaders, faith-based groups and civil society helped reach migrants and underserved populations. UNICEF emphasized sustained multisectoral engagement, continuous IPC training and tailored communication driven by behavioural insights.

UNICEF stressed that long-term catch-up requires strong digital and logistics systems. Mapping of digital tools [DHIS2 variants, U-WIN, electronic Vaccine Intelligence Network (eVIN), electronic logistics management information system (eLMIS)] and lessons from EVM assessments showed progress but also persistent cold chain gaps, stockouts and fragmented platforms. The recommended direction is interoperability, GIS-enabled planning and investments in solar infrastructure and waste management to embed catch-up within routine immunization.

Country examples illustrated practical approaches:

- Nepal benefited from policy changes, strong social drivers, the Full Immunization Declaration system and integration of catch-up into national campaigns. Future opportunities include digital registries.
- Myanmar received support across microplanning, logistics, digital payments, social mobilization and joint monitoring. Priorities include routine catch-up at every routine immunization (RI) session, stronger defaulter tracking, individual registries and expanded solar cold chain.

Across both countries, challenges persist: missed children, subnational inequities, misinformation, data gaps, accountability issues, conflict-related barriers and last-mile logistics constraints. UNICEF's priorities to "finish strong" include institutionalizing catch-up schedules, updating DHIS2 to record delayed doses, training workers on screening and multiple injections, and embedding innovative demand strategies into routine systems. UNICEF concluded by positioning immunization as a core primary health care platform and sharing resources – BeSD tools, zero-dose strategies, gender analyses – to support sustained, evidence-driven action.

Gavi perspective on country support under Gavi 6.0 strategy

Gavi presented outlines of the transition from Gavi 5.0 to Gavi 6.0, emphasizing a stronger focus on routine immunization, reducing zero-dose children, and improving countries' long-term programmatic and financial sustainability. The 2026–2030 strategy is built on equity, sustainability, gender responsiveness, climate considerations and innovation.

A key feature is the shift to a simplified, integrated funding model. Under Gavi 6.0, multiple grants – Health System Strengthening (HSS), Equity Accelerator Fund (EAF), Cold Chain Equipment Optimization Platform (CCEOP), Targeted Country Assistance

(TCA), vaccine introductions, campaigns and innovation top-ups – will be consolidated into a single five-year application managed through a unified Country Portal. This aims to reduce administrative burden, improve flexibility, and better align cash and vaccine support.

Programmatic priorities focus on strengthening routine immunization, reaching zero-dose and under-immunized communities, expanding second-year-of-life platforms and enabling routine delivery of vaccines beyond early childhood. Support will be differentiated by country context, including self-financing, transition and fragile settings. Updated policies include a revised eligibility threshold (US$ 2300 GNI per capita from 2026) and updated co-financing requirements.

A major reform is the introduction of Country Vaccine Budgets (CVB), which set a predefined ceiling for vaccine support, combining guaranteed funding for core vaccines with flexible funds for priority introductions or campaigns. Complementing this is the Vaccine Portfolio Optimization and Prioritization (VPOP) system, designed to help countries make evidence-based decisions on vaccine introductions, costs and long-term planning.

Gavi emphasized stronger integration with primary health care, improved data and digital systems, and close collaboration among ministries, national immunization technical advisory groups (NITAGs), WHO, UNICEF and partners. The strategy promotes innovation and digitalization to enhance last-mile delivery and overall immunization performance.

Gavi concluded that Gavi 6.0 is intended to be more predictable, flexible and country-driven, supporting national leadership while strengthening routine immunization, protecting vulnerable populations and ensuring resilient systems. This new architecture aligns well with BCU objectives and provides a platform for countries to institutionalize catch-up vaccination efforts.

Barriers to RI coverage, equity and service delivery (focus on urban/hard-to-reach populations) – ITSU India

The ITSU presented a data-driven overview of key immunization barriers in India through the lens of coverage, equity and service delivery, grounded in the scale of India's Universal Immunization Programme (UIP – 13 million annual sessions, 25.7 million children, 29 million pregnant women and approximately 29 000 cold chain points). While national coverage has improved since the pandemic, analysis of 2019–2024 WUENIC data showed persistent subnational inequities, especially between urban and rural populations. Urban full immunization coverage has improved more slowly, reflecting unique challenges in densely populated and mobile city environments. District-level

disparities remain pronounced, with most districts below 50% coverage concentrated in the northeast.

A major focus was India's Mission Indradhanush (MI) and Intensified Mission Indradhanush (IMI), which together vaccinated more than 54.6 million children and 13.2 million pregnant women. IMI 5.0 expanded eligibility to under-fives and stressed measles–rubella vaccination. The ITSU highlighted MI/IMI's strengths: detailed headcounts, due lists, targeted outreach, flexible session timings and strong monitoring – crucial for identifying zero-dose and under-immunized children.

Key barriers in urban areas include fragmented service delivery, weak private sector reporting, overburdened sites, limited use of platforms such as Urban Health and Nutrition Days (UHNDs), low social cohesion and highly mobile populations. Hard-to-reach regions face terrain-related disruptions, limited infrastructure, workforce shortages, long distances to cold chain points, transport challenges and data gaps from poor connectivity. Misinformation and hesitancy, amplified by social media, were noted as growing obstacles requiring strengthened communication and trusted community engagement.

In closing, the ITSU stressed that India's historically rural-focused immunization system must adapt to rapid urbanization, projected to reach nearly half the population by 2050. Sustaining progress will require addressing structural inequities, strengthening urban immunization strategies, improving data and service delivery in underserved areas, and expanding community-focused approaches to reach persistently missed populations.

Session 3: Field visit

The field visit component of the BCU review workshop provided delegates with an immersive, on-the-ground perspective of Delhi's routine immunization operations, enabling participants to observe firsthand how the city organized and delivered immunization services, from cold chain management to community-level outreach. Conducted on Day 2 of the workshop, the field visits were designed to complement the technical sessions with practical insights into high-density urban immunization systems and their approaches to catch-up vaccination under India's catch-up immunization framework.

The participants were divided into three groups and they visited sites across North-East Delhi, South-East Delhi and West Delhi, reflecting the diversity of service delivery conditions across the city. Each itinerary was structured to cover outreach immunization sessions, health facility–based services and state-level cold chain operations, followed by a consolidated technical briefing at the Directorate of Family Welfare.

Observations at immunization session sites:

Across all three field locations, delegates witnessed the organization and flow of routine immunization sessions in community settings such as Anganwadi centres and urban outreach points. Teams observed:

- session site setup and workflow, including beneficiary registration, counselling, vaccine administration, post-vaccination observation; and documentation;
- community mobilization practices led by Accredited Social Health Activists (ASHAs) and Anganwadi workers (frontline community worker under the Integrated Child Development Services), who guided caregivers through the process and reinforced immunization awareness;
- data recording methods, such as tally sheets, Mother and Child Protection (MCP) cards and due-lists used to track zero-dose and partially immunized children; and
- waste management protocols, including the use of safety boxes, segregation practices and disposal mechanisms aligned with national guidelines.

The visits highlighted the critical role of frontline workers in ensuring session quality, managing large caseloads and maintaining an enabling environment for caregivers.

Health facility visits and cold chain management

The participants visited health facilities in each district – such as the polyclinic Gautampuri (North-East), Maternity & Child Welfare Centre, Gautampuri (South-East), and Seed Primary Urban Health Centre (SPUHC), Nihal Vihar (West) – to understand how the backbone of the immunization system supports outreach activities. These facilities demonstrated:

- cold chain management practices, including storage, temperature monitoring, distribution planning and adherence to SOPs;
- staffing structures, with vaccinators and auxiliary nurse midwives (ANMs) supported by ASHAs and AWWs to cover large catchment populations; and
- session planning, where teams manage multiple monthly outreach sessions to ensure no urban or peri-urban pocket remains unserved.

In North-East Delhi, the visit also included a stop at the State Cold Chain Store, where participants reviewed vaccine storage systems, movement logs and stock management processes that support the wider Delhi immunization network.

Use of digital tools and data systems

An important learning element was the demonstration of U-WIN, India's national digital immunization platform. Through the briefing at the Directorate of Family Welfare, participants gained clarity on how U-WIN supports:

- beneficiary registration and unique health IDs;
- digital immunization records;
- session planning and micro-logistics; and
- real-time reporting and follow-up reminders.

Participants also learnt how U-WIN integrates with other digital tools such as eVIN to enhance transparency, accountability and efficiency in vaccine management.

Integration of catch-up vaccination

At every site, the participants observed how Delhi integrates catch-up vaccination into routine sessions, with frontline workers screening all children for missed doses and administering eligible vaccines following the national delayed-schedule guidelines. The city's strategies – including frequent outreach sessions, coordination among health workers and community engagement – showcased a robust system for reducing zero-dose and under-immunized children.

Key takeaways from the field visit

The field visits allowed participants to identify replicable practices, such as:

- strong coordination among ANMs, ASHAs and Anganwadi workers;
- structured microplanning and daily review mechanisms;
- effective use of digital applications for tracking and reporting;
- integrated service delivery models, combining outreach and fixed-site immunization; and
- clear catch-up protocols embedded into routine services.

Participants were encouraged to document insights relevant to their national contexts, noting innovations, operational efficiencies and community engagement approaches that could strengthen their own immunization systems.

Delhi's field visits showed a well-organized, community-driven and digitally supported immunization system where catch-up activities are smoothly integrated into routine services. Frontline workers maintained strong engagement in high-risk and mobile settlements, while session sites were efficiently managed despite limited space.

Digital tools such as U-WIN and eVIN strengthened tracking and logistics, though some data inconsistencies remain. Effective microplanning, community partnerships and regular supervision supported high coverage, with notable best practices such as remote temperature monitoring, incentives for ASHAs and colour-coded mapping. Continued focus on improving data coherence and sustaining community outreach will help maintain and enhance Delhi's strong immunization performance.

Session 4: Big Catch-Up data and reporting

BCU immunization data: Recording, reporting, quality assessment, triangulation, challenges – WHO-SEARO

The BCU immunization data session provided a clear overview of how countries currently record, report and use catch-up vaccination data, while highlighting the structural gaps that must be addressed to fully institutionalize BCU within routine systems. Participants were taken through existing data streams – primarily the electronic Joint Reporting Form (eJRF) and national BCU files – and the limitations posed by restricted antigen and cohort disaggregation, which hinder longitudinal and antigen-specific analyses.

Examples illustrated how well-structured, segregated BCU data can meaningfully correct national coverage trends, as seen in retrospective WUENIC estimations. The session also underscored the absence of standardized reporting formats, limited visibility of subnational systems and varying degrees of integration with HMIS/HIS/DHIS2 platforms.

Countries reflected on key data quality risks, including duplication between BCU and routine RI, challenges in disaggregating doses by antigen and cohort, weak triangulation with logistics and surveillance data, and denominator inconsistencies. To address these issues, the presentation introduced WHO tools – such as the data quality assessment (DQA) toolkit, Data Handbook, data quality self-assessment (DQS) and triangulation frameworks – to strengthen assessment, improve denominator accuracy and embed regular review processes.

The discussion concluded with practical implications for institutionalization: the need to design data structures for disaggregation, maintain linked yet distinct reporting streams, establish routine triangulation cycles, and invest in frontline capacity for accurate BCU recording and digital entry.

Overall, the session emphasized that achieving sustainable catch-up vaccination depends on data systems that are well-structured, clearly separated from routine RI and consistently triangulated to generate reliable insights for national decision-making.

Innovations and technologies for improving immunization programmes – PATH

PATH presented a comprehensive overview of the organization's global reach, technological innovation and longstanding role in strengthening immunization systems, with particular relevance for accelerating catch-up vaccination in the SE Asia Region. Highlighting its mission to advance health equity through innovation and partnerships, PATH showcased its five core domains – vaccines, health systems, drugs, diagnostics and devices – and its unique end-to-end capabilities spanning vaccine discovery, development, introduction and scale-up.

The participants learned how PATH's innovations, from vaccine vial monitors and Uniject devices to advanced cold chain technologies and life-saving maternal health tools, have reached millions in low-resource settings. The presentation emphasized persistent challenges in reaching zero-dose and under-immunized children, noting both supply-side barriers such as limited sessions, stockouts and uneven supervision, and demand-side issues including low awareness, social norms, misinformation, gender constraints and population mobility.

PATH outlined a holistic approach combining microplanning, training, logistics, surveillance, safety, waste management and accountability with strong community engagement and behaviour change strategies. Forward-looking segments highlighted breakthrough technologies – including MR microarray patches, freeze-preventive carriers and next-generation device innovations – supported by clinical trials and field evaluations.

PATH also showcased extensive regional contributions across SE Asia Region countries, spanning campaign monitoring, cold chain modernization, human-centred design initiatives and device testing. The presentation concluded by reaffirming PATH's commitment to advancing innovation, strengthening system resilience and driving equity-focused partnerships with WHO, UNICEF, Gavi and national governments to bring life-saving vaccines closer to underserved communities.

Localization in catch-up activities – JSI

The John Snow India (JSI) presentation highlighted localization as a powerful strategy for advancing India's Big Catch-Up efforts by shifting decision-making and problem-solving to community-level actors who best understand local contexts.

Emphasizing India's diverse and hard-to-reach populations, the JSI outlined a four-pillar community-centred model – localization, partnerships, strategic communication and innovation – to strengthen routine and catch-up immunization. Through structured NGO partnerships, hyper-localized communication and service delivery innovations such as mobile units, pop-up sessions and tribal outreach, the JSI demonstrated how

communities can be effectively mobilized to reach zero-dose and persistently missed children.

Digital tools such as HelloVAXI, GIS mapping and model immunization centres further enhanced planning and service quality. Reported outcomes – including reaching millions through communication, facilitation of over a million RI doses facilitated and engagement of thousands of community influencers – showcased the impact of community-anchored systems.

The JSI concluded that localization is essential for sustainable, equitable immunization systems, driven by collaboration, empowered local networks and deep community trust.

Accelerating disease control and elimination – WHO-SEARO

WHO-SEARO presented a forward-looking overview of how the WHO SE Asia Region is advancing vaccine-preventable disease control under the Regional Vaccine Implementation Plan (2022–2026), aligned with IA2030. It highlighted the Region's three impact goals – equitable access, life-course vaccination and stronger systems – and reviewed mixed progress towards elimination targets, noting sustained polio-free and maternal and neonatal tetanus elimination (MNTE) status, expanded rubella elimination and setbacks in measles due to immunity gaps.

The presentation outlined five pillars for acceleration: high population immunity, strong surveillance, robust laboratories, outbreak preparedness and supportive policies. Data showed reductions in zero-dose children, rising life-course coverage and increased new vaccine introductions, while emphasizing persistent inequities. Key vaccines and their coverage challenges – such as low hepatitis B (HepB) birth dose uptake and incomplete IPV2 rollout – were discussed alongside regional performance trends.

The presentation underscored that high coverage alone does not guarantee population immunity, requiring both prospective (routine strengthening, boosters, second-year platforms) and retrospective (catch-up campaigns, SIAs, periodic intensification of routine immunization) strategies.

It concluded with a comprehensive framework built on maximizing reach, strengthening programme management, mobilizing communities and rigorous monitoring, aligning closely with the objectives of the Big Catch-Up initiative.

Global update on the Big Catch-Up – WHO HQ

WHO HQ presented a global update on the BCU initiative and underscored the importance of strong monitoring, evaluation and learning systems to institutionalize routine catch-up vaccination. The session introduced the BCU monitoring dashboard and

outlined WHO's three-part monitoring, evaluation and learning (MEL) framework – country monitoring, outcome evaluation and a learning agenda – to address common challenges such as eligibility screening, documenting older-age catch-up doses and maintaining the integrity of routine administrative data.

Drawing on case studies from multiple countries, WHO highlighted how BCU efforts successfully reached previously missed children, strengthened stakeholder coordination, and improved tools such as SOPs, schedules and recording systems. However, reliance on multiround, campaign-style approaches was noted as unsustainable for long-term catch-up, especially when dealing with multiple antigens and operational constraints.

Looking ahead, the presentation emphasized the need to embed catch-up within routine services, address systemic barriers such as staffing and financing, and support decentralization so that subnational teams can own and resource ongoing efforts.

WHO HQ concluded with key resources – training materials, reporting guidance, missed opportunities for vaccination (MOV) tools and planning documents – to help countries integrate routine catch-up, strengthen MEL systems, improve data tools, empower providers to reduce missed opportunities and align national reporting with global monitoring requirements.

Session 5: Group work on Country Action Plan (2026)

The Country Action Plan session showcased how Nepal, Timor-Leste and Myanmar are moving beyond the Big Catch-Up to institutionalize routine catch-up immunization through stronger systems, community engagement and data-driven strategies.

Nepal highlighted its robust policy foundation, including the Immunization Act and ward-level coordination, that has enabled catch-up to be embedded within routine services, supported by digital microplanning, refined risk stratification and innovative community mobilization approaches.

Timor-Leste emphasized a three-tier approach spanning strong policies, programme strengthening and hyper-local implementation using community leaders, volunteers, digital EPI registers and the PIS platform, reinforced by quarterly RI cycles and efforts to reduce missed opportunities.

Myanmar outlined plans to maximize reach through added fixed-post days, integrated routine and catch-up services, digital mapping of mobile populations, revitalized volunteer networks and expanded multilingual social mobilization, supported by stronger monitoring and domestic financing. Across all three countries, common themes included embedding catch-up within routine immunization, leveraging digital tools

for tracking missed children, mobilizing community networks, addressing challenges posed by mobility and urbanization, and investing in local ownership and sustainable resources to maintain progress.

Closing session

The closing session synthesized three days of field insights, country presentations and technical discussions into a shared commitment to institutionalizing routine catch-up immunization across the Region.

Participants acknowledged significant progress under the BCU initiative, including improvements in routine coverage, reductions in zero-dose children, strengthened planning and monitoring, and the successful integration of catch-up practices in countries such as India, Nepal and Myanmar. Country teams highlighted common enablers – targeted microplanning, innovative delivery models, strong community engagement and expanded use of digital tools – while also recognizing persistent challenges such as urban and migrant inequities, data quality gaps, workforce and financing constraints, and the difficulty of transitioning from campaign-style momentum to sustained routine delivery.

Building on these reflections, Dr Suman Rijal, Director of the Department of Communicable Diseases, WHO-SEARO, underscored that catch-up immunization must become a routine, institutionalized component of national programmes, with particular attention to vulnerable populations including socially disadvantaged communities, urban slum dwellers and migrants. He acknowledged ongoing resource constraints but reaffirmed that immunization remains one of the most cost-effective public health investments, calling for sustained advocacy, innovation, integrated delivery, strong community engagement, and greater use of digital tools and monitoring systems. Appreciating the rich cross-country learning generated through presentations and field visits, he reaffirmed WHO and UNICEF's continued commitment to collaboration and support.

The session concluded with a collective agreement to embed catch-up within all routine contacts, strengthen policies and SOPs, improve digital reporting systems, enhance district-level monitoring and advance dose reconciliation efforts. Countries agreed to refine and validate their action plans post-workshop, guided by the principle that "catch-up must become the norm, not an exception".

5. Annexures

Annexure 1. Agenda

Day 1 – Wednesday, 10 December 2025

Opening

- Registration
- Welcome remarks
- Opening remarks
- Objectives and briefing on agenda
- Introduction of participants.

Session 1: Country achievements on Big Catch-Up (2024–2025)

- India – Achievements, challenges and best practices
- Myanmar – Achievements, challenges and best practices
- Nepal – Achievements, challenges and best practices
- Timor-Leste – Achievements, challenges and best practices.

Session 2: Overview and partner perspectives

- Overview of Big Catch-Up and institutionalizing catch-up immunization: WHO SEARO
- UNICEF insights on implementation and strengthening routine immunization
- Gavi perspective on country support under Gavi 6.0 Strategy
- Barriers to RI coverage, equity and service delivery (focus on urban/hard-to-reach populations) – ITSU India.

Session 3: Field visit orientation

- Orientation on team composition, session sites, tools, methodology and movement plan.

Day 2 – Thursday, 11 December 2025

Field Visit

- Visit to facility for cold chain and data reporting
- Visit to outreach session sites
- Travel to Directorate of Family Welfare (Vikas Bhawan-II)

- Briefing on routine immunization in Delhi
    - Use of digital applications (U-WIN)
    - Special immunization sessions.

Day 3 – Friday, 12 December 2025

Reflection and presentations

- Team work to consolidate field visit findings
- Group presentations and discussion.

Session 4: Big Catch-Up data and reporting

- BCU immunization data: recording, reporting, quality assessment, triangulation challenges
- Innovations and technologies for improving immunization programmes – PATH
- Localization in catch-up activities – JSI
- Accelerating disease control and elimination – WHO SEARO
- Global update on the Big Catch-Up – WHO HQ.

Session 5: Group work on Country Action Plan (2026)

- Group work to develop action plans for reducing zero-dose children
- Presentation of country action plans (priorities for 2026 and beyond).

Closing

- Closing session – IVD Coordinator, WHO SEARO.

Annexure 2. List of participants

Ministry of Health (National EPI Programme)

1. Mr Sanjay Kumar Mahaseth
   Public Health Officer
   Department of Health Services
   Ministry of Health and Population
   Teku, Nepal

2. Dr Angelina da Costa Rangel Barreto
   Medical Officer for EPI
   Ministry of Health
   Dili, Timor-Leste

3. Dr Aida dos Santos Simoes
   EPI Data Management Officer
   Ministry of Health
   Dili, Timor-Leste

Ministry of Health (Subnational EPI Programme)

4. Mr Bharat Bhandari
   Immunization Officer
   Health Service Directorate
   Ministry of Health and Population
   Koshi Province, Nepal

5. Mr Uttam Raj Pyakurel
   Vector Control Supervisor
   Health Directorate
   Ministry of Health and Population
   Bagmati Province, Nepal

6. Mr Lakshmeshwar Jha
   Immunization Supervisor Officer
   Health Directorate
   Ministry of Health and Population
   Madhesh Province, Nepal

7. Mr Ek Narayan Lamsal
   Immunization Supervisor Officer Health Directorate, Ministry of Health and Population
   Gandaki Province, Nepal

8. Mr Minraj Joshi, Public Health Inspector Health Directorate, Ministry of Health and Population
   Sudurpaschim Province, Nepal

9. Mr Kumar Thapa, Family Planning Inspector Health Directorate, Ministry of Health and Population
   Lumbini Province, Nepal

10. Mr Om Prasad Paudel Public Health Officer Health Service Directorate, Ministry of Health and Population
    Karnali Province, Nepal

11. Dr Adelaide Maria Tilman
    EPI DPHO Viqueque
    Municipal Health Services, Ministry of Health
    Dili, Timor-Leste

12. Dr Antonia da Silva
    EPI DPHO Viqueque
    Municipal Health Services, Ministry of Health
    Dili, Timor-Leste

Immunization experts

13. Dr Kyaw Kan Kaung
    Immunization Expert, NITAG
    Yangon, Myanmar

14. Dr Toe Thiri Aung
    Immunization Expert, NITAG
    Yangon, Myanmar

15. Dr Tin Thitsa Lwin
    Immunization Expert, NITAG
    Yangon, Myanmar

Donors and partners

Gavi, the Vaccine Alliance

16. Dr Nilgun Aydogan (virtual)
    Senior Country Manager
    Geneva, Switzerland

Immunization Technical Support Unit (ITSU)

17. Dr Pritu Dhalaria
    Director
    New Delhi, India

John Snow India (JSI)

18. Dr Gopal Soni Project
    Director, RISE and V-Shiksha Project
    New Delhi, India

Programme for Appropriate Technology in Health (PATH)

19. Dr Ankur Mutreja
    Director, External Affairs and Health Security
    New Delhi, India

20. Dr Achintya Srivatsa
    PATH
    New Delhi, India

UNICEF Regional Office for South-Asia (ROSA)

21. Ms Hilary Adams
    Regional Immunization Specialist
    Kathmandu, Nepal

UNICEF Country Offices

22. Dr Hsu Myat Myo Naing
    Health Specialist (EPI)
    Yangon, Myanmar

23. Mr Prakash Chandra Joshi
    Immunization Officer
    Kathmandu, Nepal

World Health Organization

WHO Country Offices

24. Dr Asish Satapathy
    National Professional Officer, Deputy Team Lead (NPSP)
    New Delhi, India

25. Dr Vishesh Kumar
    National Professional Officer, Immunization
    New Delhi, India

26. Dr Raveesha Mugali
    Medical Officer (EPI & VPD Surveillance)
    Yangon, Myanmar

27. Dr Yee Yee Cho
    National Professional Officer
    Yangon, Myanmar

28. Dr Zin Bo Bo Htet
    National Technical Officer (Immunization)
    Yangon, Myanmar

29. Ms Lei Lei Mon
    Data Management Associate
    Yangon, Myanmar

30. Dr Balwinder Singh Chawla
    Medical Officer
    Kathmandu, Nepal

31. Dr Rahul Pradhan
    National Professional Officer
    Kathmandu, Nepal

32. Dr Nitesh Kumar Mishra
SMO Madhesh
Kathmandu, Nepal

33. Mr Mateus Cunha
National Professional Officer
Dili, Timor-Leste

WHO-SEARO

34. Dr Suman Rijal
Director
Communicable Diseases Department
New Delhi, India

35. Dr Vinod Kumar Bura
Coordinator
Immunization and Vaccine Development
New Delhi, India

36. Dr Sudhir Khanal
Regional Advisor (Accelerated Disease Control)
Immunization and Vaccine Development
New Delhi, India

37. Dr Rahul Srivastava
Technical Officer (Measles and VPD Research)
Immunization and Vaccine Development
New Delhi, India

38. Dr Tondo Opute Emmanuel Njambe
Technical Officer (New Vaccines Introduction)
Immunization and Vaccine Development
New Delhi, India

39. Dr Kim Kwang Jin (Virtual)
Technical Officer (Health Systems Strengthening)
Immunization and Vaccine Development
New Delhi, India

40. Mr Raman Sethi
Data Management Officer
Immunization and Vaccine Development
New Delhi, India

41. Ms Monica Chellaramani
Executive Assistant
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New Delhi, India

42. Ms Balreen Kaur
Executive Assistant
Immunization and Vaccine Development
New Delhi, India

Stills from the workshop

Subnational review workshop on
Big Catch-up initiative
in priority countries of the
WHO South-East Asia Region
10–12 December 2025, New Delhi, India
Gavi

STATE VACCINE STORE
DIRECTORATE OF FAMILY WELFARE
GOVERNMENT OF (NCT) DELHI